

LATOM Wellness Protocol: GI Health & Gut Optimization

A patient guide to finding the root cause of bloating, brain fog, food sensitivities, and chronic gut issues

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For: \$10 paywalled protocol PDF on latomwellness.com

WARNING: This protocol is educational. It does not establish a practitioner-patient relationship. For personalized recommendations, book a consultation at latomwellness.com.

Why your gut matters more than most doctors tell you

Your gut is doing more than digesting food. It hosts roughly 70 percent of your immune system, manufactures most of your serotonin, and trains your body to recognize what is food versus what is a threat. When gut function breaks down, the downstream signs often look unrelated: brain fog, anxiety, joint pain, skin issues, fatigue, weight stuck.

Conventional medicine is good at finding gut disease (Crohn's, ulcerative colitis, celiac, cancer). It is less good at finding gut dysfunction. Most patients told they have "IBS" are told there is nothing structurally wrong, here is a fiber recommendation and an antispasmodic. That diagnosis often misses fixable causes.

This protocol walks through the labs that actually find root causes, the supplements with real evidence, when prescription medication makes sense as a bridge, and when to escalate to a gastroenterologist.

The labs that find root causes

1. GI-MAP (DIAGNOSTIC SOLUTIONS)

A comprehensive PCR-based stool test. Reports:

- Pathogens: *H. pylori* (with virulence factors), *C. difficile*, *Salmonella*, *Shigella*, *Cryptosporidium*, *Giardia*, and more
- Opportunistic bacteria: *Klebsiella*, *Citrobacter*, *Pseudomonas*, and others that overgrow when commensals decline
- Commensal bacteria: the "good" bacteria including *Bifidobacterium*, *Lactobacillus*

- Yeast and parasites: Candida species, Blastocystis hominis, Entamoeba
- Inflammation markers: calprotectin, beta-glucuronidase, secretory IgA
- Digestive markers: elastase-1, steatocrit (fat malabsorption)
- Zonulin (intestinal permeability marker)

This is the single most useful test for chronic GI symptoms in patients without a clear structural diagnosis. Patient cost via Fullscript runs roughly \$300 to \$400 depending on shipping and processing. (Verify current pricing in your Fullscript order.)

2. CELIAC SCREEN: TTG-IGA + TOTAL IGA

Tissue transglutaminase IgA is the standard first-line screening test for celiac disease per the American College of Gastroenterology 2023 guidelines. Total IgA is drawn at the same time because roughly 1 in 40 patients are IgA-deficient, which would make the TTG-IgA falsely negative.

Important: You must be eating gluten regularly for at least 6 weeks before this test, or it will be falsely negative. If you have already eliminated gluten, either restart gluten for 6 weeks (gluten challenge) or skip the antibody test and proceed to endoscopy.

(Source: Rubio-Tapia A, et al. American College of Gastroenterology Guideline: Diagnosis and Management of Celiac Disease. Am J Gastroenterol. 2023;118:59-76.)

3. CALPROTECTIN (STOOL)

A non-invasive marker that distinguishes inflammatory bowel disease (Crohn's, ulcerative colitis) from irritable bowel syndrome. Calprotectin is elevated when neutrophils are infiltrating the gut wall.

- Below 50 mcg/g: IBD highly unlikely
- 50 to 150 mcg/g: borderline, repeat in 4 to 6 weeks
- Above 150 mcg/g: refer to gastroenterology for colonoscopy

(Source: AGA Clinical Practice Guidelines on the role of biomarkers for the diagnosis and management of IBD. Gastroenterology. 2023.)

4. SIBO BREATH TEST (LACTULOSE OR GLUCOSE)

Small intestinal bacterial overgrowth is the underlying mechanism for a significant portion of "IBS-D" and "IBS-C" diagnoses. Glucose breath test is more specific for proximal SIBO. Lactulose breath test captures distal SIBO including methane-dominant patterns (which often present as constipation).

Methane-positive SIBO (often called intestinal methanogen overgrowth or IMO) responds to different antibiotic regimens than hydrogen-positive SIBO.

5. STANDARD ADJUNCTS

- Comprehensive metabolic panel: liver and kidney function, electrolytes
- Complete blood count: anemia (iron-deficiency from chronic GI bleeding is a red flag)
- Ferritin and iron panel: low ferritin in a patient with GI symptoms is a meaningful signal
- Vitamin D, B12 with methylmalonic acid: malabsorption screen

- Thyroid full panel: TSH plus free T3, free T4, reverse T3 (thyroid affects motility)
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Supplement protocol by GI pattern

WARNING: Evidence levels for each supplement are categorized. Strong evidence supplements are well-supported in clinical trials. Moderate evidence supplements have mechanistic rationale and mixed trial data. Use selectively. Always check for drug-supplement interactions with any medications you take.

PATTERN A: HEARTBURN, REFLUX, INDIGESTION (GERD PATTERN)

Strong evidence

- Deglycyrrhizinated licorice (DGL) 380 to 760 mg chewed 20 minutes before meals. Soothes mucosa, supports the protective layer over the stomach lining. Removed glycyrrhizin to avoid blood pressure effects. (Source: Larijani VN, et al. Evaluation of the effect of an Olive leaf extract and DGL in non-erosive reflux disease. J Gastroenterol Hepatol Res. 2014;3(7):1224-1227.)
- Zinc carnosine 75 mg twice daily. Heals gastric mucosa, supports H. pylori treatment. (Source: Mahmood A, et al. Zinc carnosine, a health food supplement that stabilises small bowel integrity and stimulates gut repair processes. Gut. 2007;56(2):168-175.)
- Magnesium glycinate 400 mg before bed if you also experience constipation

Moderate evidence

- N-acetylcysteine (NAC) 600 to 1200 mg daily. Reduces mucus viscosity, antioxidant support, may help if reflux is paired with sinus or respiratory symptoms.
- Slippery elm or marshmallow root tea: traditional soothing mucilage, limited modern trials but well-tolerated and centuries of empirical use

Prescription medication option: a PPI course

If symptoms are significant and lab workup is in progress, an 8 to 12 week course of a proton pump inhibitor (omeprazole 20 to 40 mg daily, or equivalent) is reasonable as a bridge. PPIs are not meant for indefinite use. Long-term PPI use carries risks including bone density loss, B12 malabsorption, and increased C. difficile susceptibility.

(Source: AGA Clinical Practice Update on PPI deprescribing. Gastroenterology. 2022;162:1287-1305.)

The right sequence is usually: PPI for symptom relief during workup, then taper as supplement protocol and lifestyle changes take effect.

PATTERN B: BLOATING, GAS, ABDOMINAL DISTENSION (SIBO/IBS PATTERN)

Strong evidence

- Berberine 500 mg three times daily for 4 weeks. Natural antimicrobial effect, similar in efficacy to rifaximin in small trials for SIBO. (Source: Chedid V, et al. Herbal therapy is equivalent to rifaximin for the treatment of small intestinal bacterial overgrowth. Glob Adv Health Med. 2014;3(3):16-24.)

- Peppermint oil enteric-coated 180 to 200 mg three times daily. Smooth muscle relaxation, modest but well-replicated benefit for IBS.
- Magnesium citrate 400 mg if predominantly constipated

Moderate evidence

- *Saccharomyces boulardii* (probiotic yeast) 5 billion CFU twice daily. Distinct from bacterial probiotics. Supports treatment of dysbiosis and antibiotic-associated diarrhea.
- L-glutamine 5 to 10 g daily on empty stomach. Primary fuel for enterocytes (small intestinal cells).
- Digestive enzymes (pancreatin or plant-based) with each meal if bloating is post-prandial

Prescription option (if SIBO confirmed)

Rifaximin (550 mg three times daily for 14 days) is the first-line antibiotic for hydrogen-positive SIBO. For methane-positive SIBO, the combination of rifaximin plus neomycin (or rifaximin plus metronidazole) is standard.

PATTERN C: DIARRHEA-PREDOMINANT PATTERN

Strong evidence

- *Saccharomyces boulardii* 5 billion CFU twice daily
- Soluble fiber (psyllium husk or partially hydrolyzed guar gum, PHGG/Sunfiber) 5 to 10 g daily. Note: PHGG is gentler than psyllium for sensitive guts.

Rule out first

- Bile acid malabsorption (consider cholestyramine trial)
- Microscopic colitis (requires colonoscopy with random biopsies)
- Celiac (TTG-IgA + total IgA, as above)
- IBD (calprotectin)

PATTERN D: CONSTIPATION-PREDOMINANT PATTERN

Strong evidence

- Magnesium citrate or magnesium oxide 400 to 800 mg at bedtime, titrate to soft stool
- Vitamin C buffered 2 to 4 g daily, titrate to bowel tolerance
- Soluble fiber (psyllium or PHGG) 5 to 10 g daily

Moderate evidence

- Probiotic with *Bifidobacterium lactis* strains
- Castor oil packs externally for severe constipation
- Triphala 1 g at bedtime (Ayurvedic, used for centuries, modest modern data)

Prescription options for refractory constipation

- Linaclotide (Linzess), lubiprostone (Amitiza), prucalopride (Motegrity) - see your physician.

PATTERN E: MUCOSAL HEALING (ACROSS PATTERNS)

These are the foundational supplements that support gut wall integrity across most patterns:

- L-glutamine 5 to 10 g daily
 - Zinc carnosine 75 mg twice daily
 - Vitamin D3 5000 IU daily (ensure 25-OH level above 50 ng/mL)
 - Omega-3 EPA/DHA 2 to 4 g daily (anti-inflammatory)
 - Bovine colostrum 1 to 2 g daily (immunoglobulin and growth factor support)
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Lifestyle protocol

Supplements without lifestyle change rarely fix chronic gut issues. The following are non-optional:

DIET

- Low-FODMAP elimination for 4 to 6 weeks if bloating is dominant, then systematic reintroduction
- Mediterranean pattern as the long-term default (high fiber, polyphenols, omega-3, fermented foods)
- Eliminate alcohol for at least 30 days during workup (impairs healing)
- Eliminate gluten and dairy for 30 days as a trial if not already done (note: avoid this BEFORE celiac testing)
- Slow down eating, chew thoroughly, sit while eating

STRESS AND THE GUT-BRAIN AXIS

Chronic stress directly damages the gut barrier through cortisol-mediated mechanisms and disrupts motility. Address stress with the same seriousness as diet.

- Vagal tone work: humming, gargling, cold water face immersion, controlled breathing
- Daily 10 to 20 minute walk after each meal (improves motility)
- 7 to 9 hours of sleep, dark cool quiet bedroom
- One screens-off hour before bed

SLEEP AND THE MIGRATING MOTOR COMPLEX

Your gut cleans itself between meals through a wave called the migrating motor complex (MMC). The MMC is suppressed by frequent eating. Eating one meal every 3 hours kills the MMC. Stretches of 4 to 5 hours between meals allow it to run.

MOVEMENT

A daily 10-minute walk after dinner outperforms many supplements for digestive function.

When to see a gastroenterologist

This protocol is meant to support gut health, not replace gastroenterology when it's needed. Refer urgently if any of these are present:

- Blood in stool (bright red or melena) - same day urgent
- Unintentional weight loss greater than 5 percent body weight in 6 months
- Persistent vomiting
- Difficulty swallowing
- Severe abdominal pain especially with fever
- Family history of colon cancer plus new GI symptoms (especially if first-degree relative diagnosed before age 50)
- Iron-deficiency anemia of unclear cause
- Age 45 or older without ever having had a screening colonoscopy
- New persistent change in bowel habits over age 50

LATOM does not replace gastroenterology. If you have any of the above, your first call is to a gastroenterologist for a colonoscopy or upper endoscopy. A LATOM consultation can complement that workup but does not substitute for it.

How to work with LATOM on this

STEP 1: BOOK A CONSULTATION

A 30-minute video visit. We review your symptoms, history, prior labs, and current medications.

STEP 2: WE ORDER TARGETED LABS

Usually a GI-MAP plus the celiac and calprotectin screens, plus a blood panel covering CBC, CMP, thyroid, vitamin D, B12, ferritin. Labs are billed through our partner network at wholesale pricing.

STEP 3: WE BUILD YOUR PROTOCOL

Based on results, we build a stepwise plan including:

- A supplement stack from our Fullscript dispensary (15 to 30 percent off retail, practitioner-curated)
- A dietary protocol matched to your pattern
- A lifestyle and stress plan
- Where indicated, a short prescription bridge (PPI, rifaximin, or other)
- Referral coordination to gastroenterology if needed

STEP 4: FOLLOW-UP

We see you back in 6 weeks to assess response and adjust. Most patients with chronic GI symptoms have meaningful improvement by week 8 to 12 when the workup catches the root cause.

Cost expectations

Item	Cost range
LATOM consultation (initial)	\$149
GI-MAP test	\$300 to \$400
Celiac TTG-IgA + total IgA (LabCorp through Fullscript)	\$40 to \$80
Calprotectin (LabCorp through Fullscript)	\$80 to \$120
Basic blood panel (CBC, CMP, thyroid, D, B12, ferritin)	\$80 to \$150
Supplement starter stack for 30 days	\$80 to \$200 depending on pattern
Total first 30 days	\$730 to \$1,100

Compare to: insurance copays for the same GI workup typically run \$400 to \$1,200 out of pocket after annual deductible, often without the GI-MAP or full thyroid panel included.

What this protocol is NOT

- Not a substitute for emergency care if you have red flag symptoms
- Not a replacement for gastroenterology, oncology, or surgical consultation when indicated
- Not appropriate during active pregnancy without OB approval (some supplements should be avoided)
- Not a way to skip a recommended colonoscopy if you are due for one
- Not a guarantee that all GI symptoms can be fixed without prescription medication

Sources

This protocol cites peer-reviewed clinical literature and professional society guidelines. Key references:

1. Rubio-Tapia A, et al. American College of Gastroenterology Guideline: Diagnosis and Management of Celiac Disease. Am J Gastroenterol. 2023;118:59-76.
2. AGA Clinical Practice Guidelines on the role of biomarkers for the diagnosis and management of IBD. Gastroenterology. 2023.
3. Chedid V, Dhillon S, et al. Herbal therapy is equivalent to rifaximin for the treatment of small intestinal bacterial overgrowth. Glob Adv Health Med. 2014;3(3):16-24.
4. Mahmood A, et al. Zinc carnosine, a health food supplement that stabilises small bowel integrity and stimulates gut repair processes. Gut. 2007;56(2):168-175.
5. Larijani VN, et al. Evaluation of the effect of an Olive leaf extract and DGL in non-erosive reflux disease. J Gastroenterol Hepatol Res. 2014;3(7):1224-1227.

6. AGA Clinical Practice Update on PPI deprescribing. *Gastroenterology*. 2022;162:1287-1305.

7. Hayes PA, Fraher MH, Quigley EM. Irritable bowel syndrome: the role of food in pathogenesis and management. *Gastroenterol Hepatol (N Y)*. 2014;10(3):164-174.

This educational guide is provided by LATOM Wellness, PLLC. For personalized care, book a consultation at latomwellness.com.

Information in this protocol reflects clinical evidence as of 2026-05-24 and is subject to change as new research emerges. Re-verify dosing, contraindications, and drug interactions with your prescribing physician.